Standard Operational Procedures

MCH Clinic

This document (SOP) serves as a tool for the staff to improve the quality of health care delivery with a view to increase patients' satisfaction. Thus, everyone must follow and comply with it.

Prepared By: HNL

MCH Clinic

The Maternal and Child Clinic has been organized to provide services for both well and sick children from ages 0 month to 14 years old and for postpartum mothers. The specific services include: 1.) A well-baby clinic that comprises of newborn assessment/health screening for 2 weeks, 6 weeks, 2months, 4 months, 6months, 9 months, 12months, and a yearly follow-up until a child is 5 years old: 2.) Administration of immunization to all age: 3.) A Pediatric Clinic that comprised of sick children who required pediatric medical evaluation, and those who are discharged from Emergency Room and Pediatric Unit and are require follow-up care. 4.) Pediatric High Risk Clinic and Children with Special Health Care Needs, 5) Minor treatment and procedures such as breathing treatments, dressing changes irrigation, IV/IM/PO meds medication administration and etc., are send to OPD or ER.

The clinic has service providers such as pediatricians, registered nurse, practical nurse, nursing assistants, and data encoder, with coordinated services for pharmacy, radiology, laboratory, Dental, and other Public Health Clinics.

The clinic has the services accessible to patients on Monday through Friday, form 8:00-4:30, on appointment and walk-in basis. Any patient who comes to the clinic after 4:30 will be advised to go the emergency room for treatment. The Saturday immunization clinic for immunization update is open from 8:00-11:30 noon, on appointment basis. Clinic is flexible to accept walk-ins but to a minimum on that day. Clinic is closed on Sundays and holidays.

Standard Operational Procedure No. 1

Process:	Reporting to Duty
Objective:	To appear professionally when entering working station; and to provide and serve the patients on a timely manner
Responsibility	MCH staff

No.	Procedures	Standard
1.	Punctuality	• All staff should report to duty 5-10 minutes prior to time of duty 8:00 AM – 12:00 PM Morning Clinic 1:00 PM – 5:00 PM Afternoon Clinic • All staff should punch in and out using the biometric machine
2.	Absent from duty	• Staff should immediately notify his/her immediate supervisor, if he/she is unable to come to work • Staff should provide a leave slip to his/her supervisor on the day of absent or one day prior to her/his leave; or no leave will be granted
3.	Grooming	• All staff shall follow the MOH dress code • Female nurse: - Blue top, and navy blue or black skirt • Male nurse - Blue top, and navy blue or black pant • should wear cleaned and ironed uniform • Only cleaned white cover shoes are worn • Female nurse • Hair has to be neatly tied and above shoulder line • Male nurse are not allowed to wear long hair • All nurses should use mild perfume/deodorants • All nurses should wear a wrist watch • Always wear your beautiful and handsome smile

Standard Operational Procedure No. 2

Process:	Processing and Handling of Patients Medical Record Chart
Objective:	To ensure that patient's records are properly documented
Responsibility	All

No.	Procedures	Standard
1.	Process of Medical Record Chart	• All patients must proceed to medical record department to process their medical chart and proceed to MCH patient waiting area for the triage nurse to call them • Medical record staff should bring patients' chart to the MCH Triage room • All children 0 – 5 years must present with their yellow card to the medical record department. Those children without yellow card must see the immunization clerk to get a copy of a yellow card.
2.	Recording and documenting	• All patients information should be properly documented in patients chart
3.	Returning of Medical Charts	• Charge nurse and clerk must collect all charts, tally all encounter for that day and transfer data to the daily encounter log book • Nurse should ensure that all charts are return to medical record department at the end of each clinic day • No charts are to be kept in the clinic unless needed by doctors

Standard Operational Procedure No. 3

Process:	Triage
Objective:	To ensure that patient needs are met in a timely manner with appropriate care
Responsibility	Triage nurse

No.	Procedures	Standard
1.	Well-baby Check-up & Immunization (8:00AM-12:00PM)	• All patients must proceed to medical record to process their medical charts then proceed to MCH waiting area for the triage to call them • Fill up and complete all necessary forms (immunization encounter form, the outpatient encounter form, & other required form) • Check vital signs and measurement <u>0-1yr</u> Patient's Temperature Patient's weight (no clothes) Patient's height Head circumference Abdomen circumference Chest circumference <u>2-5yrs</u> Patient's Temperature Patient's weight Patient's height All measurement must be in inches, weight in pounds, and temperature in Fahrenheit • All Vital signs and measurement must be recorded in patients charts • Plot the child weight in both the child yellow card and the yellow card in the medical record chart- (growth development section) • Always fill up the breast feeding section on the immunization encounter form • Direct all patients for immunization to the immunization room • All children should be seen and examined by a Pediatrician per MCH Well-Baby check-Up schedule • Refer all sick patients to pediatrician prior to immunization
2.	OPD clinic	• All patients must proceed to medical record to process

their medical charts then proceed to MCH waiting area for the triage nurse to call them

- Clinical supervisor nurse must assign a nurse to assist doctors at all time
- Take vital signs and Measurement and record in patients' charts

  Vital signs:
  - Temperature – (Fahrenheit)
  - Pulse rate – as needed
  - Respiratory rate – As needed
  - Blood Pressure – As needed

  Measurement:
  - Height/length- inches
  - Weight – pounds
- Charge nurse should make sure that surveillance section on the patient encounter form is filled up
- Prior to OPD clinic assigned nurse must collect lab results and have them ready for doctors' used – for follow up patients
- Assess patient's condition for any indication of distress or other condition that will need immediate medical attention and refer accordingly.

  Indications for immediate care include, shortness of breath, high grade fever, and signs of dehydration)
- Treat patient with high fever per protocol (refer to table below)
- Apply sponge bath if temperature is greater than 101°F

Acetaminophen: 10 – 20mg /kg/dose q 4 hours

80mg/0.8ml drop; 160mg/5ml syrup

Dose is based on 15mg/kg/dose

Weight (pound)	Weight (kg)	Dose (mg) q4h	Drop (ml)	Suggested amt. of drop	Syrup (ml)	Suggested amt. of syrup
5	2.3	34	0.3	0.4ml		
10	4.5	68	0.7	0.8ml	2.1	½ tsp
15	6.8	102	1.0		3.2	½ tsp
20	9.1	136			4.3	1 tsp
25	11.4	170			5.3	1 tsp
30	13.6	205			6.4	1 tsp
35	15.9	239			7.5	1 ½ tsp
40	18.2	273			8.5	1 ½ tsp
45	20.5	307			9.6	2 tsp
50	22.7	341			10.7	2 tsp

Standard Operational Procedure No. 4

Process:	Conducting Immunization Session
Objective:	To ensure that all children seen at the clinic are vaccinated with required immunization
Responsibility	Immunization nurse

No.	Procedures	Standard
1.	Routine vaccination	• Assess and screen patient prior to vaccination • Always check BCG result and note result on both baby's yellow card and the yellow card in the chart • Always start with the least painful vaccines or with vaccines given by mouth/nasal • Always used the "Five Rights Right patient Right drug Right dose Right route Right time • Use recommended needle size for each vaccine • Clean injection site prior to vaccination • Explain to the parents about post vaccination reaction and treatment (refer to Immunization guideline) • An appointment for next immunization visit should be given before a patient leave the clinic
2.	Adult/high risk group	• All patient for vaccination should come with an outpatient encounter form • All required and recommended vaccine should be administered on site • Record all vaccine in the outpatient encounter and the immunization encounter form
3.	walk-in/referring patient	• Make sure patients has his/her outpatient encounter form before vaccination is given • Check or mark all vaccination given on both outpatient encounter form and the immunization encounter form • All patients that missed their appointment date must be referred to zone nurses for follow up • All postpartum mothers and other mothers should referred to Family Planning Nurse on site for family planning counselling

3.	Recording and documenting	• Outpatient encounter • Record all given vaccines in the immunization and outpatient encounter forms, on the patient yellow card, and in the medical record chart • Submit all immunization encounter form to the data clerk to enter in the Immunization registry (WEBIZ) • All immunization data must be enter at the end of each clinic day • All appointment for immunization must be logged in the immunization appointment log book
4.	Issuing out immunization verification/yellow card	• All children 0-5 years will need their chart from medical record for immunization update before issuing a new verification/yellow card • Birth certificate must present before processing of immunization verification/yellow card • Any lost yellow card will only be issued out when a receipt of payment is presented to MCH staff – Payment is made at Health Care Revenue Office

Standard Operational Procedure No. 5

Process:	Breast feeding
Objective:	To ensure that all mothers have full understanding of the importance of breast-feeding-exclusively
Responsibility	MCH nurse/pediatrician

No.	Procedures	Standard
1.	Breast feeding Education	• Breast feeding education must be provided to all mothers on the 2 weeks visit (postnatal visit) • Nutrition counselling to mothers as needed • IEC on Breast feeding will distributed during the postnatal clinic • Video showing on breast feeding during postnatal clinic
2.	Recording and Reporting	• Triage nurse must collect data on breast feeding on all children 0-6 months of age (breast feeding section on the immunization encounter form) • Triage charge nurse must record breast feeding data on the breast feeding log book that is place in the triage room • Triage charge nurse should compile data and submit report to Clinical Chief Nurse on a monthly basis

Standard Operational Procedure No. 6

Process:	Nebulization treatment
Objective:	To ensure safe and proper administration of nebulizer treatment
Responsibility	MCH Nurse

<table>
<tr><th>No.</th><th>Procedures</th><th>Standard</th></tr>
<tr><td>1.</td><td>Medication supply</td><td>
<ul>
<li>Assigned nurse should make sure medication and other needed supplies are available at all times
<u>Supply needed:</u>
- Albuterol inhalation 5mg/ml
- Albuterol sulfate inhalation solution 2.5mg/3ml Or whichever is on stock
- Normal Saline
- Syringes
- Needles
- Clean nebulization tubing
- Working nebulizer machine</li>
</ul>
</td></tr>
<tr><td>2.</td><td>Medication Administration/Doses</td><td>
<ul>
<li>Nebulization treatment should ONLY administered to a patient when indicated by doctors</li>
<li>Always follow nebulization treatment guideline</li>
</ul>
<u>Guideline:</u>
<table>
<tr><td>Age</td><td>mg</td><td>ml</td><td>NSS</td></tr>
<tr><td>Children 6 yrs to 12 yrs</td><td>2.5mg – 5mg</td><td>0.5ml</td><td>2ml</td></tr>
<tr><td>Below 6yrs</td><td>1.25ml</td><td>0.25ml</td><td>2.5ml</td></tr>
</table>
Note: Treatment can be repeat 3-4 times daily
<ul>
<li>Do not exceed recommended dose</li>
<li>Refer patients if no improvement</li>
</ul>
</td></tr>
<tr><td>3.</td><td>Recording</td><td>
<ul>
<li>Always record and document in patient chart</li>
<li>Record treatment in the nebulization log book</li>
</ul>
</td></tr>
</table>

Standard Operational Procedure No. 7

Process:	Patient admission
Objective:	Admitting patient occurs on a timely and safe manner
Responsibility	MCH Nurse

No.	Procedures	Standard
1.	Admitting physician roles	• Complete all doctors' admitting order • Notify assigned nurse • Admitting physician may perform procedures, (e.g IV/Heplock insertion, blood extraction & etc.) in difficult cases
2.	Nurse roles	• Review doctor's admission order • Notify the Unit about patient admission Information to provide to the admitting unit - Patient Age and gender - Reason for admission - And other information • Request lab tests • Initial IV and po meds should be given at MCH unless fail to put in IV line • Make sure all forms and admission note are completed before transferring patient to the admitting UNIT • Patient's admission information is then enter into the admission log book • Transfer patient and endorsed patient to the UNIT nurse • Clean and dispose all used supplies in a proper disposing technique

Standard Operational Procedure No. 8

Process:	Maintained a clean and safe working environment
Objective:	To ensure clean instrument are used for the patient and safe working environment
Responsibility	Nurse; Immunization program manager

No.	Procedures	Standard
1.	Disinfect and cleaning of medical equipment	• Thermoscan thermometer must be clean with ONLY isopropyl or ethyl alcohol at the end of the day and as needed • All Diagnostic set in doctors exam room must be clean with alcohol or disinfectant solution at the end of a working day and as needed • Examination Bed are to be clean at the end of a working day and as needed
2.	Exam rooms	• Nurse must clean doctors' exam room after each working day • Nurse must ensure that exam room are stock with needed medical supply and forms
3.	Clean-Up	• Cleaning and caring for MCH, is everyone responsibility and all staff must do their part • Assigned staff must follow the clean-up schedule • Always use disinfectant solution for cleaning
4.	MCH Vehicle	• Program manager should ensure outreach vehicle are maintain regularly

Standard Operational Procedure No.9

Process:	Administering Vitamin A and Dewoming
Objective:	To ensure safe and timely administration of Vitamin A and deworming
Responsibility	All nurse

<table>
<tr><th>No.</th><th>Procedures</th><th>Standard</th></tr>
<tr><td>1.</td><td>Distributing and Administering</td><td>
<ul>
<li>Administer twice a year
1st round March
2nd round September</li>
<li>Assigned nurse must ensure that clinic is well stocked</li>
<li>Give vitamin A to all 6mos-14 years during the scheduled month (at the clinic, zones, outer islands)</li>
<li>Give deworming to all 2 years to 14 years per recommended guideline</li>
</ul>
</td></tr>
<tr><td>2.</td><td>Recording and reporting</td><td>
<ul>
<li>Vitamin A and Deworming administration should marked in child yellow card</li>
<li>Data are recorded into the Vitamin A and dworming Tally sheet</li>
<li>Clinical charge nurse should collect, compile data and submit to clinical chief nurse – (monthly)</li>
<li>Same process apply at the zones and outer islands outreach</li>
</ul>
</td></tr>
<tr><td>3.</td><td>Recommended Vitamin A and deworming dosing</td><td>
<ul>
<li>Nurse must follow WHO guideline below:</li>
</ul>
Suggested Vitamin A supplementation scheme for children 6-59 months of age
<table>
<tr><td>Target group</td><td>6mos-11mos</td><td>12mos-59mos</td></tr>
<tr><td>Dose</td><td>100,000 U</td><td>200,000 U</td></tr>
<tr><td>Route</td><td>Oral</td><td>Oral</td></tr>
</table>
Deworming (Albendazole 400mg)
<table>
<tr><td>Target Age group</td><td>1yr</td><td>2yrs and up</td></tr>
<tr><td>Dose</td><td>200mg (1/2 tab)</td><td>400mg (1tab)</td></tr>
<tr><td>Route</td><td>Oral</td><td>Oral</td></tr>
</table>
Deworming (Pyrantel 25omg/5ml)
<table>
<tr><td>Target Age group</td><td>Less than 5yrs</td><td>5yrs to 9yrs</td><td>10yrs to 14yrs</td><td>15yrs and up</td></tr>
<tr><td>Dose</td><td>0.5 tsp</td><td>400mg (1tab)</td><td></td><td></td></tr>
</table>
</td></tr>
</table>

Standard Operational Procedure No.10

Process:	Immunization Session in the Community
Objective:	To ensure safe vaccine administration and to avoid missed opportunities
Responsibility	Zone nurses, cold-chain staff

No.	Procedures	Standard
1.	Preparation	• All needed supplies should be prepared a day before an immunization session is conducted <u>Needed Supplies</u>: - syringes 3cc - needles 25G 1', G25 ½ ', - BCG syringes - Gloves all sizes - Safety box - Trash bags - Hand sanitizer - Alcohol pad - Forms (encounter forms, zone worksheet, blank yellow card, adult immunization work sheet) - Tables and chairs - Lap top - Educational hand outs • Zone nurse will prepare and submit vaccines request form to cold-chain staff one day before the session • Zone nurse must obtain defaulter list from data clerk • Cold chain staff should prepare and have Vaccines carrier ready • Zone nurse must make sure they have all supplies before they are loaded on the outreach vehicle
2.	During immunization session	• Prior to the session, the nurse must organized their working area • Nurse must record all needed vaccines on the yellow card and in the immunization encounter form before a child get vaccinated • The nurse should Disseminate immunization handouts to mothers • Vaccinator must apply safety technique when

		vaccinating • Zone nurse must make sure all required forms are filled up • Zone nurse must check and make sure vaccine temperature are always maintain recommended range
3.	After the session	• Zone nurse must clean the area and make sure no used needles and syringes are left on the ground • Zone nurse must tabulate doses administration report and enter to the vaccine request form • Zone nurse will return all unused vaccines to the cold chain nurse who will then count them and enter it to the vaccine request form • Zone nurse must clean the vaccine carrier and restock the outreach bag for next session • Zone nurse must make sure all used syringes and needles are properly disposed • Zone nurse must clean the vehicle and make sure no used needles, syringes and gloves are left in the vehicle

Standard Operational Procedure No.11

Process:	In-house rules
Objective:	To have a safe, clean and a positive working environment for the staff and the patients
Responsibility	MCH staff

Standard
• Respect others • Be a team player • Come to work on time • Always keep your supervisor aware • Give every patient your total attention, patience and courtesy • No **CHEWING BETELNUT** • All staff shall follow and comply with **Tobacco Policy** • No eating in the examination rooms • All staff must follow their assigned schedule • Always wear gloves during patient contact • Always apply protective personal equipment (PPE) • Nurses, always practice **"NO RECAP**" tech • Hand washing before and after each patient contact and as needed • **BE ECONOMICAL**

Approved by: Herokko Neamon, RN Date: _____
CN, Public Health Clinic

Dr. Jane Gancio, Pediatrician Date: _____
Department Head, Pediatric

Dr. Aina Garstang, MO Date: _____
Director, Public Health

Dr. Tom Jack, MO Date: _____

Mailynn K. Lang Date: _____
Deputy Secretary, MOH

Dr. Kenner Briand, MBBS Date: _____
Secretary of Health